



Public Health Emergency Operations Centre "COUSP-DRC"

Ebola Virus Disease Situation Report/17th Outbreak

SitRep MVE No. 001/2026

Country/Province	DRC/Ituri
Affected Health Zone	Mongbwalu, Bunia and Rwampara
Reporting date	May 14, 2026
Publication date	May 15, 2026

						
261	8	246	4	?	?	?
Alert	Confirmed Cases	Suspected Cases	Deaths confirmed	Bed occupancy rates among confirmed patients	Bed occupancy rates among suspected cases	Healed

I. KEY POINTS

- ☒ 3 ZS/36 are affected including Bunia, Rwampara and Mongbwalu.
- ☒ 8 confirmed cases, including 4 deaths.
- ☒ The second meeting of the national emergency coordination was held with all stakeholders, chaired by the Director General of the INSP, regarding the MVE_ITURI situation.



Context

The COUSP/DRC received an alert reporting a group of deaths of unknown etiology in the Mongbwalu Health Zone (90 km from Bunia), Djugu territory in the Ituri province, which is located in the eastern part of the Democratic Republic of Congo.

A preliminary health alert dated May 5, 2026, originated from healthcare personnel in the Mongbwalu health zone and was officially reported on May 8 following reports of unexplained cluster deaths (50 deaths via social media). Between April 1 and May 13, 2026, 261–280 suspected and probable cases, including 100 deaths (55 in-hospital and 45 community deaths in the Mongbwalu health zone), were reported, including 4 deaths, all of whom were healthcare professionals. All these cases originated from the Mongbwalu, Rwampara, and Bunia health zones. The outbreak is located in the Shun (or Sun 1) neighborhood of Mongbwalu (Alimasi 1 and 2 avenues).

The event reportedly occurred in a context of strong suspicion of a community epidemic, initially marked by rapid and unexplained deaths, some of which were linked to risky body handling practices and funeral rituals and prayer groups.

After reviewing in-hospital deaths, some common clinical signs such as fever, headache, vomiting, intense physical asthenia in the majority of cases and dyspnea, hematemesis, epistaxis, blackish vomit in other cases were noted.

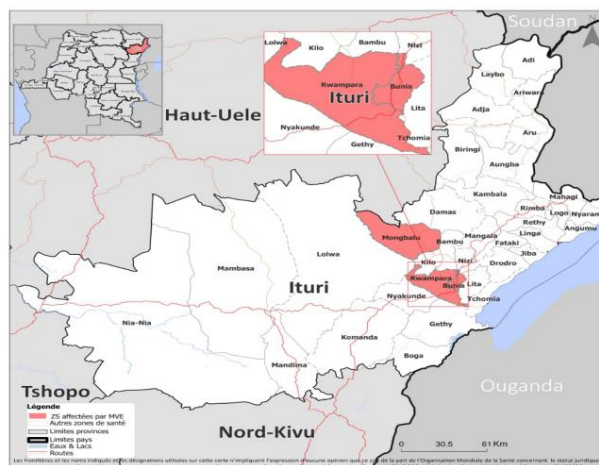
Investigations conducted by the EIR on [date] resulted in the collection of 28 samples from suspected cases. Initially, preliminary tests performed at INRB/Bunia were all negative for Ebola Zaire, dengue, rotavirus, cholera, malaria, Yersinia pestis, MPOX, and COVID-19 before 13 of these samples were sent to INRB/Kinshasa.

As of May 14, 2026, eight cases tested positive for Ebola virus disease (Bundibugyo/Uganda strain) at the INRB in Kinshasa. This will be the 17th Ebola virus disease outbreak in the DRC.

II. EPIDEMIOLOGICAL SITUATION UPDATE

On April 1st and May 13th, 2026, 246 suspected cases, including 80 deaths, were reported, among which 4 deaths were healthcare professionals. All these cases originated from the Health Zones (HZs) of Mongbwalu, Rwampara, and Bunia. The main hotspot is located in the Shun (or Sun 1) district of Mongbwalu, particularly on Alimasi 1 and 2 avenues.

Indicateur	Valeur
Cas confirmés Ebola	8
Échantillons à reprendre	4
Cas suspects/probables estimés	246 246
Décès hospitaliers HGR Mongbwalu	55
Décès communautaires Mongbwalu	2
Personnel de santé décédé	≥40
Contacts identifiés	≥82
ZS affectées	3



l'importance des décès ; la rapidité d'évolution ; l'atteinte des soignants ; et les formes hémorragiques observées ; suggèrent une létalité élevée compatible avec une flambée sévère de MVE.

III. COORDINATION

- ☒ Holding of national coordination meetings with all stakeholders and these provinces.
- ☒ Preparation of the MVE_17 ITURI response plan to COUSP in response mode
- ☒ Preparation for the activation of EIRs in cascade and equipment
- ☒ Preparing for the deployment of teams at all levels

IV. EPIDEMIOLOGICAL SURVEILLANCE

- ☒ Official notification of the alert on May 8, 2026 following rumors of group deaths.
- ☒ Preliminary investigations have traced the origin of the event back to the Shun district (Mongbwalu) as early as May 5th.
- ☒ Investigation of 3 validated and sampled alerts,
- ☒ Holding of a meeting with 5 ITs and ECZ on the active search for community and intrahospital deaths, Pre-listing
- ☒ of contacts (63 contacts) and remaining to be completed,
- ☒ Strengthening of active surveillance in 3 AS (CECA 20, Mongbwalu and Abelkoko),
- ☒ Development of contact lists to initiate tracing.
- ☒ Active search for suspected FHA cases at Mongbwalu Hospital,
- ☒ securing bodies

V. LABORATORY

- ☒ Thirteen initial samples taken, including 2 analyzed by RDT with the result negative (Dengue, MPOX, Malaria, Yesinia pestis, COVID-19, Rotavirus and Cholera), Biological samples taken from
- ☒ suspected cases of the 28 samples and sent to INRB/Kinshasa for analysis.

VII. INFECTION PREVENTION AND CONTROL (PCI/WASH)

- ☒ PCI evaluation, CS Abelkoko (score 7%), HGR Mongbwalu (score 34%)
- ☒ PCI briefing: hand hygiene, waste management and patient isolation y Isolation preparation at HGR Mongbwalu Implementation of Infection
- ☒ Prevention and Control (PCI) measures in local healthcare facilities, despite implementation difficulties.

IX. RISK COMMUNICATION AND COMMUNITY ENGAGEMENT

- ☒ Initial awareness-raising of community and religious leaders to counter rumors of "mystical illness".

X. SECURITY

- ☒ Daily security monitoring.

CHALLENGES AND GAPS

- ☒ Lack of compliant isolation, development of the Individualized Care Plan (PAI) and designation of the Integrated Care Service (SGI)
- ☒ Lack of PCI supplies, sampling kits
- ☒ CREC needs to manage community rumors.
- ☒ Personnel not trained in infection prevention and control (IPC), sample collection, transport and storage, surveillance, and management of diseases with epidemic potential
- ☒ Lack of triage at HGR Mongbwalu and CS
- ☒ Complete EDS implementation
- ☒ Urgent need for standardized CTE in the 3 ZS during epidemics

XIII. PERSPECTIVES

- ☑ Organize and standardize Dignified and Safe Burials (DSBs) to neutralize the major source of contamination represented by bodies.
- ☑ Install or strengthen Ebola Treatment Centers (ETCs) for the early management of patients.
- ☑ Intensify community engagement to stop mass collective prayers and risky funeral rituals.
- ☑ Resolve the crisis of confidence between the Health Zone and the population by providing transparent information on biological confirmations.
- ☑ Initiate immediate cross-border coordination with Uganda and South Sudan to monitor population movements at the borders. Activate the Public Health Emergency Operations Centre (PHEOC) through the National Institute of Public Health (NIPS) in response mode, level 1 according to the operational concept.
- ☑ A call for the mobilization of technical and financial partners to demonstrate solidarity and alleviate the suffering of the people behind the action of the government of the republic as well said: **A coordination team, A plan and A budget ;**
- ☑ Strengthening epidemiological and biological surveillance in affected and at-risk health zones, providing holistic
- ☑ and free care for Ebola cases and implementing both rapid and strict isolation of suspected or confirmed patients,
- ☑ Strengthening communication about the risks of the disease and community engagement against this disease in a context of worrying security, high population growth, and significant population mobility,
- ☑ The immediate deployment of rapid response teams (RRTs) in a cascading fashion to protect and save human lives. Likewise, the deployment of essential medicines, safe blood, therapeutic molecules, and laboratory reagents and equipment cannot be delayed.

exercise area ;

ILLUSTRATION



Coordination meeting with the Director General of the INSP



latest events PROVINCIAL COORDINATION



FOR ALL
INFORMATION
ADDITIONAL,
PLEASE CONTACT

For the National Institute of Public Health (INSP) of the DRC

The Director General of INSP

Dr. MWAMBA KAZADI Dieudonné

Tel.: +243 816 040 145

Email: diedonnmwambakazadi@gmail.com

The COU-SP Coordinator

Professor NGANDU Christian

Tel.: +243998091915

Email: nganduchristian@ymail.com

The Incident Manager SGI MVE Epi17

Email:

For the World Health Organization (WHO/DRC)

The WHO Representative in the DRC

Dr

Email: @who.int

WHO Incident Manager in the Ebola Response

Email: @who.int

The WHO EPR Cluster Lead Team in the DRC

Dr. Mouctar Diallo

Email: dialloam@who.int

